



ANZCA
FPM

AUSTRALIAN AND NEW ZEALAND COLLEGE OF ANAESTHETISTS

2025.2 PRIMARY EXAMINATION REPORT

GENERAL COMMENTS

This report is provided to candidates, Supervisors of Training, and tutors, to assist them with exam preparation.

The ANZCA primary examination assesses knowledge and understanding of the applied basic sciences, which are the foundation of clinical anaesthesia. It is set at a level appropriate to postgraduate specialist medical training.

Appendix 2 of the ANZCA Curriculum sets out the Learning Outcomes (LOs) which may be examined in the primary examination. **Appendix 2 should form the basis of teaching and study programs to prepare for the primary examination.**

Candidates must allow sufficient time to prepare – typically, approximately 12 months of study is required in order to cover all of the LOs in Appendix 2.

The list of recommended textbooks is set out on the ANZCA library website. Some topics will require additional research, and candidates are encouraged to read widely. However non-peer-reviewed resources (books or websites) should be used with caution.

Planning for success, was launched in late 2024 on the Learn@ANZCA website. It is an online exam preparation resource provided by ANZCA and designed to support exam candidates. It was developed by examiners, SOTs, performance psychologists and successful trainees to help prospective candidates gain an understanding of the exam expectations and format, and suggested strategies in preparation. It is highly recommended for anyone approaching the Primary Exam.

Prior to undertaking the exam, candidates are strongly encouraged to discuss their level of preparedness with local tutors. This should include performance in trial written and viva questions.

Past SAQs are a valuable learning resource, but they should not be used as a 'syllabus'. This approach is likely to create significant gaps in learning, leaving candidates poorly prepared to answer new (or modified) questions. Candidates should not aim to memorise a bank of questions with the expectation that these will appear on each paper. Instead, past SAQs should be used to – revise material already studied; test understanding; practise structuring information to answer a specific question; and practise answering questions within a time limit.

The comments made in the exam reports about individual questions are not intended to represent model answers, and should not be read as such. It is not implied that all points mentioned are necessary for a pass.

Candidates are advised to review the ANZCA Academic Integrity Policy, which applies to all aspects of this examination.

OVERALL EXAM INFORMATION

This information is provided to allow unsuccessful candidates to assess their performance against that of their peer cohort. When used in conjunction with the individual feedback sent to unsuccessful candidates an appraisal of an individual's relative strengths and weaknesses can be made.

It is also provided to allow comparison of overall exam statistics across time.

Exam Statistics

MCQ paper	invitation $\geq 103/150$ questions	190 / 253 (75%)
<i>37 candidates carried an MCQ invitation from a previous exam</i>		
SAQ paper	passed $\geq 50\%$	129 / 290 (44%)
	invitation score $\geq 40\%$	231 / 290 (80%)
invited to vivas		213 / 290 (73%)
viva examination	passed $\geq 50\%$	192 / 213 (90%)
successful invitees	SAQ + viva $\Sigma \geq 50\%$	175 / 213 (82%)
overall pass		175 / 290 (60.3%)

77 candidates did not receive a viva invitation. The reasons for this were –

- 45 did not meet the invitation mark for both the MCQ and SAQ
- 18 did not meet the MCQ invitation mark of 103/150 (but did reach an SAQ invitation mark)
- 14 did not meet the SAQ invitation mark of 40% (but did have an MCQ invitation mark)

For those **invited** to vivas the correlation between MCQ score and overall result is as follows;

MCQ score (out of 150) 103-106/150 had an overall pass rate of 40%.

MCQ score (out of 150) $\geq 107/150$ had an overall pass rate of 82%.

For those **invited** to vivas the correlation between SAQ score and overall result is as follows;

SAQ score (out of 75) of 30, 31 or 32 had an overall pass rate of 28%.

SAQ score (out of 75) of 35, 36 or 37 had an overall pass rate of 80%.

SAQ score (out of 75) ≥ 38 had an overall pass rate of 97%.

Candidates on their first attempt at the primary exam had an overall pass rate of 69%.

Candidates on their second attempt at the primary exam had an overall pass rate of 52%.

Renton Prize

Murari Ramesh
Roshan Selvaratnam

Merit Certificates

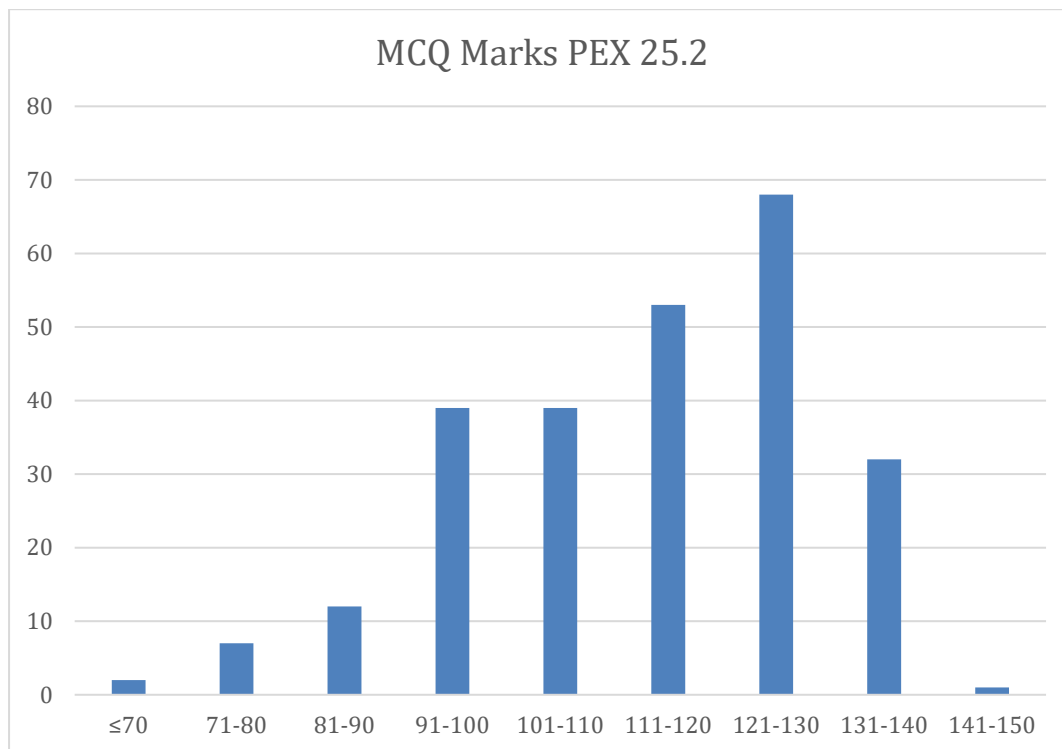
Bridget Jones
James Devin
Thomas Sizeland
Nicola Kluger
Peter Williams
Alexander Keen
Mark Rafla

MULTIPLE CHOICE (MCQ) PAPER

The MCQ paper comprises 150 questions in 150 minutes. Each question has four options, with one single best answer. There is no negative marking. Further information about the MCQ Paper is available on Learn@ANZCA in the ***Planning for Success*** online examination resource.

At this exam, the **MCQ invitation mark** was 103/150. The median score was 117/150.

The invitation mark is set prior to the examination using the Ebel method. The invitation mark varies between exams depending upon the overall difficulty of the 150 questions. The invitation mark (combined with an SAQ invitation mark of 40%) is needed to be invited to the vivas, however no marks from the MCQ contribute to the overall final score.



Group	Median score
All candidates who sat MCQ 25.2 (253)	117/150
Invitees who sat MCQ 25.2 (178)	122/150
Overall successful and sat MCQ 25.2 (149)	125/150

All questions in the MCQ paper are blueprinted to the curriculum. The MCQ paper enables the curriculum to be sampled broadly, including minor topics. For each subtopic listed below the number of questions *templated* to appear is given in the final column. In any given year the number may vary slightly due to overall question content and composition of the paper as a whole when “integrated” questions (containing overlapping learning objectives and subtopics) are included.

	2025.2	Template
anatomy	5	5
respiratory physiology	8	9
CVS physiology	8	8
nervous system physiology	9	8
renal physiology	4	4
maternal physiology	5	4
haematology and blood products	7	6
acid-base physiology	3	4
other physiology		NA
pharmacokinetics / pharmacodynamics	10	10
inhalational anaesthetic pharmacology	7	6
intravenous anaesthetic pharmacology	9	10
NMB and reversal pharmacology	5	6
local anaesthetic pharmacology	3	4
opioid pharmacology	5	5
CVS pharmacology	6	6
drugs affecting coagulation	4	4
other pharmacology		NA
monitoring / measurement / equipment	8	9
integrated		NA

AREAS OF POOR KNOWLEDGE AND UNDERSTANDING IN 2025.2 MCQ PAPER

There were several topic areas where the cohort demonstrated a poor knowledge base. These included:

- Antibiotic spectrum of activity
- Relationship between duration of effect and half-life of a drug
- Skeletal muscle physiology
- Laryngeal anatomy
- Clinical use of 3-lead ECG
- Arteriolar flow dynamics

SHORT ANSWER (SAQ) PAPER

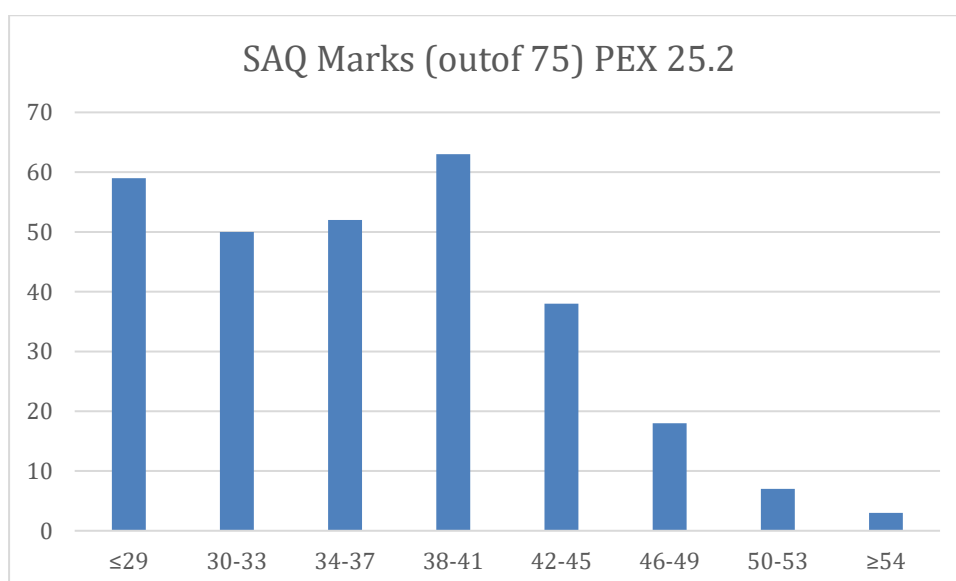
The SAQ paper comprises 15 questions in 150 minutes. There is reading time of 15 minutes, during which time notes may be made on the question paper. Each question is of equal value. Unless otherwise indicated, any sub-sections of questions are equally weighted. The marks from the SAQ paper contribute 50% of the total final score.

The pass mark for each individual question is 3/5, marked using either a holistic marking rubric (typically used for conceptual questions); or an analytical ('tally') marking rubric (typically for fact-based questions). An example of a holistic rubric is given as an Appendix to the 2019.1 exam report. The allocation of marks is based on demonstrating knowledge and understanding across a number of domains. Standard setting for the SAQ paper is achieved by varying the expectation for 3/5 for each question to match a reasonable 'minimum level of competence', depending on the topic. Errors or contradictory statements do contribute to a reduction in marks. Irrelevant information does not gain marks.

The pass mark for the SAQ paper as a whole is 50%, and the viva invitation mark is 40%. For the 25.2 exam the median scores for different groups is shown below;

Group	Median score
All candidates (290)	37/75
Invitees (213)	39/75
Overall successful (175)	40/75

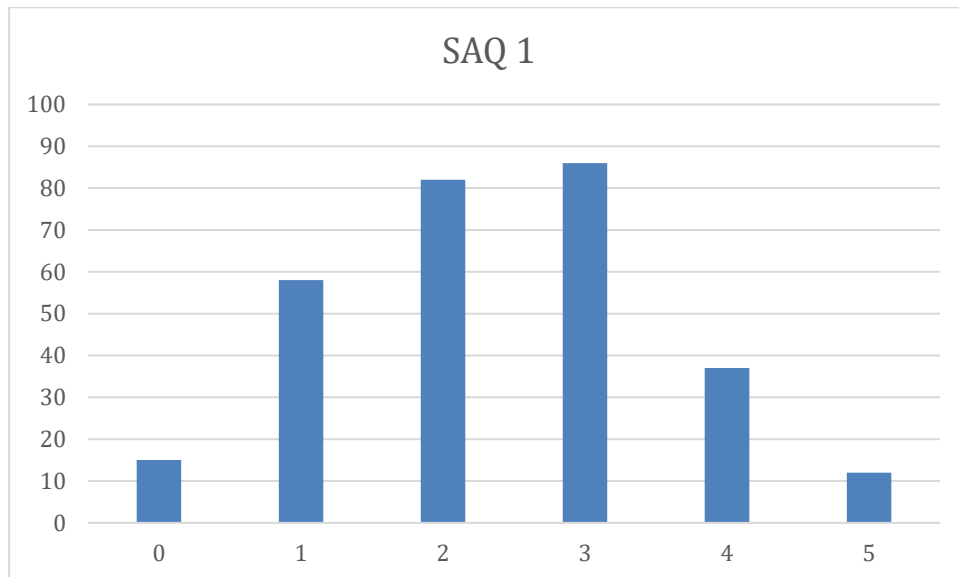
Further information about the SAQ Paper is available on Learn@ANZCA in the **Planning for Success** online examination resource, and in the previous Exam reports of 2025.1 and 2024.2. The information given in these past reports remains relevant for the current exam.



44% of candidates achieved a pass in the SAQ paper.

PERFORMANCE ON INDIVIDUAL SHORT ANSWER QUESTIONS

1. Cardiovascular physiology (left ventricular diastole) – see separate book on Learn@ANZCA



47% of candidates achieved a pass in question 1.

Learning Outcome(s)

- BT_PO 1.44 Describe the physiology of cardiac muscle ...
- BT_PO 1.44a Describe the events of the cardiac cycle

Answer domains

- Listing, describing and graphically demonstrating the phases of LV diastole
- Application of knowledge of diastole to explain the impact of AF on LV filling

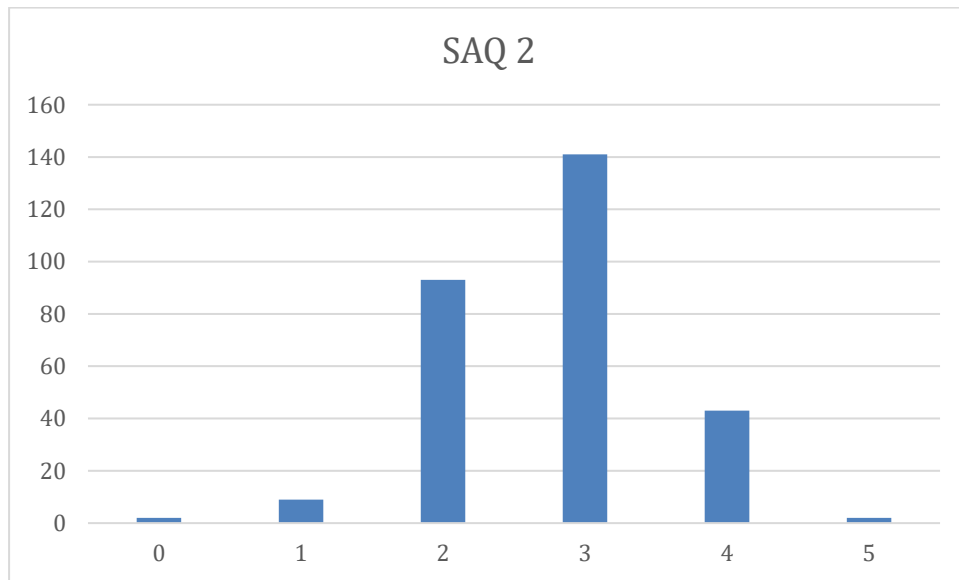
Additional credit and features of better answers

- Demonstrating the interplay between LV filling events
- Accuracy in shape and duration of diagram, and estimations of volumes and time values

Problems and recurring errors

- Shape of graph often poorly represented
- Internal inconsistencies between graph drawn and phases described
- Describing systole instead of diastole
- Omitting phases of diastole, or attributing systolic phases as diastolic phases
- Reproduction of LV pressure-volume loops with no integration with the requested elements

2. a) Draw and label an oxygen-haemoglobin dissociation curve for adult haemoglobin.
b) Why does the curve have this shape?
c) What advantages are conferred by this shape?



64% of candidates achieved a pass in question 2.

Learning Outcome(s)

BT_PO 1.31 Discuss the carriage of oxygen in blood, the oxyhaemoglobin dissociation curve, oxygen stores in the blood and their clinical significance and implications

Answer domains

- Diagram of the OHDC with labelled axes and relevant points
- Co-operative binding as a dynamic process causing the shape of the curve
- Consideration of the flat and steep parts of the curve and implications for oxygen carriage, loading and unloading

Additional credit and features of better answers

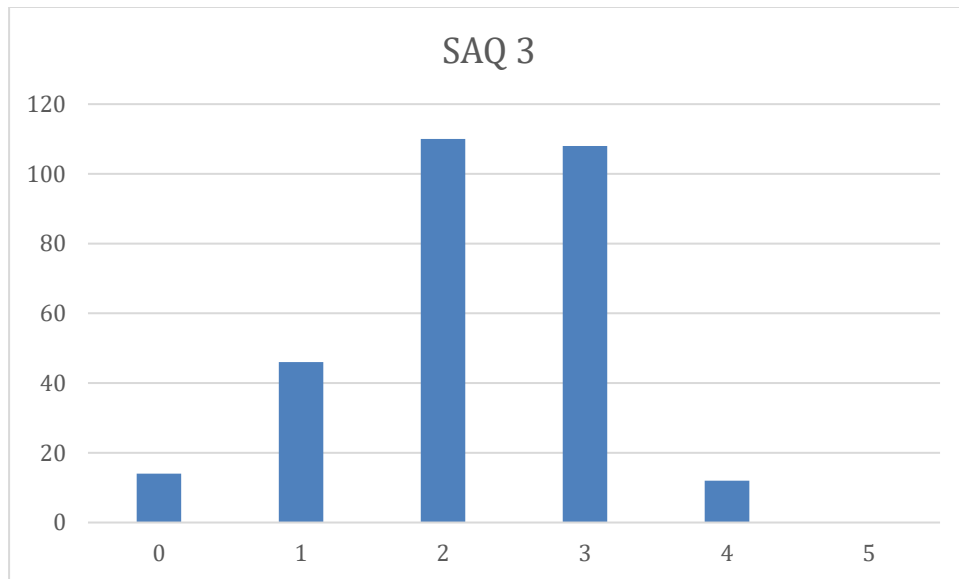
- Distinction between the venous and arterial curve
- Understanding of the factors influencing the curve shape and position, and the physiological relevance
- Details on the advantages derived from the flat and steep parts of the curve

Problems and recurring errors

- Poorly drawn curves (including wrong shape, incorrect axes and unrelated points)
- Factual recount of haemoglobin structure without linking this to the shape of the curve
- Describing the curve or its shift without discussing advantages related to its shape
- Excessive focus on dissolved oxygen

3. Outline the effects of class III obesity (BMI > 40kg/m²) on respiratory physiology using the following headings

- lung volumes
- respiratory mechanics
- V/Q relationships



41% of candidates achieved a pass in question 3.

Learning Outcome(s)

BT_PO 1.37 Discuss the effect of the following on ventilation ... obesity

Answer Domains

- Lung volumes
- Respiratory mechanics
- V/Q relationships

Additional credit and features of better answers

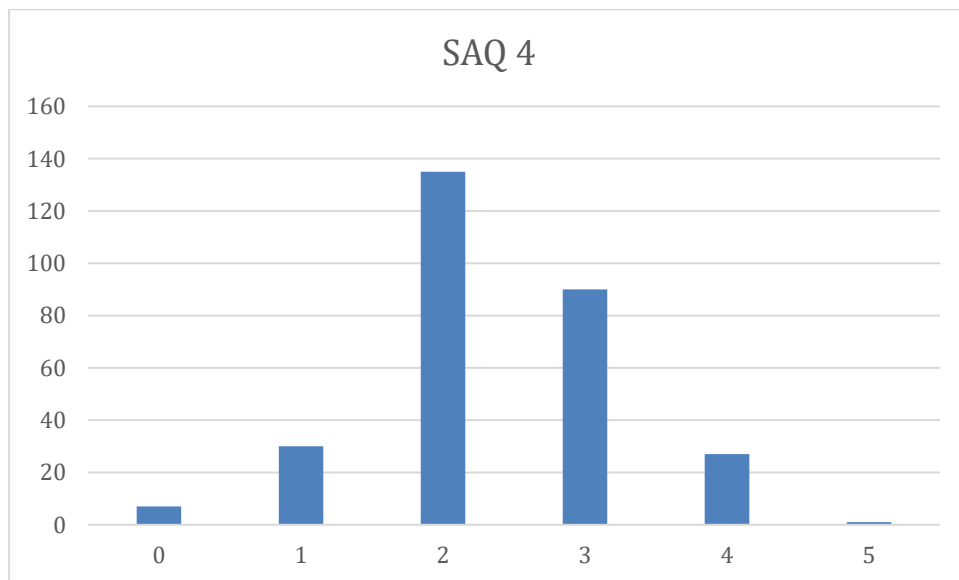
- Consideration of lung volumes other than FRC, such as closing capacity, ERV or TV
- Identification of parameters within respiratory mechanics to further categorise the answer such as chest wall compliance, lung compliance, airway resistance, and work of breathing
- Described the effects of obesity on each region of the lung based on V/Q matching

Problems and recurring errors

- Lengthy introductions including material which did not attract any marks (for example definition of class III obesity)
- Inclusion of equations and graphs which were unrelated to the domains asked
- Errors in understanding of closing capacity, and effects of obesity on lung volumes other than FRC

4. Outline the physiology of nociception using the following headings

- **stimulation of peripheral nociceptors**
- **impulse conduction along primary afferent nerve**
- **synaptic transmission at the dorsal horn**



41% of candidates achieved a pass in question 4.

Learning Outcome(s)

- | | |
|------------|--|
| BT_PM 1.3 | Describe the basic physiological mechanisms of pain... |
| BT_RA 1.1 | Describe the physiology of nerve conduction |
| BT_PO 1.92 | |

Answer Domains

- Peripheral nociception - nociceptors, stimulus for activation, signal generation.
- Impulse conduction - nerve fibre types, action potential generation and conduction, physical pathways, cell body location.
- Synaptic transmission - anatomy of dorsal horn, location and process of neurotransmission.

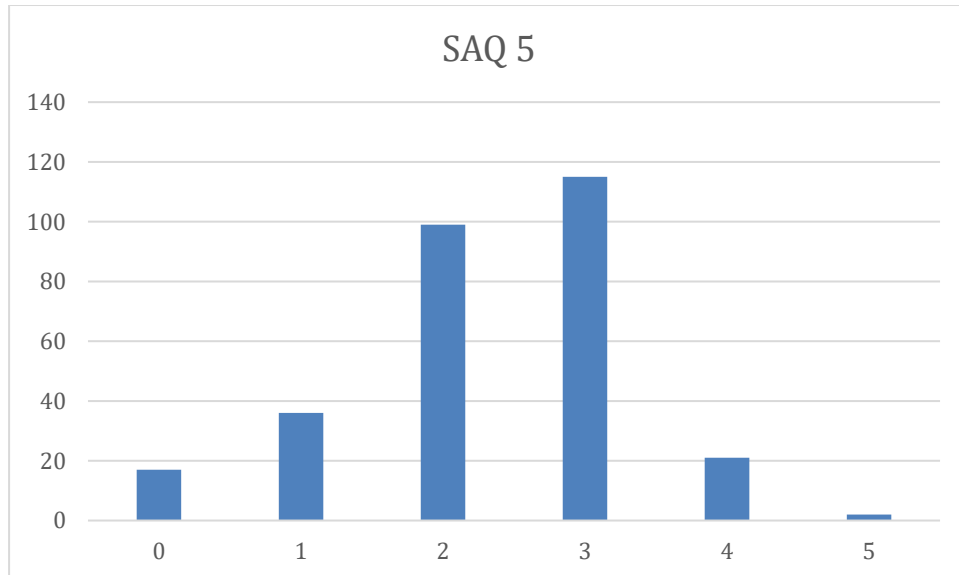
Additional credit and features of better answers

- Precise description of peripheral nociceptors – free nerve endings, details of specialised molecules and ion channels.
- Including information about mechanisms of peripheral nociception
- Details on nerve fibre types involved in nociception, AP generation, and effects of myelination on conduction speeds
- Details on dorsal horn laminae, involved neurotransmitters, and modulation mechanisms.

Problems and recurring errors

- Providing details on how drugs affect the nociceptive pathway. This did not attract marks.
- Imprecise delineation between nociception and pain in descriptions given while answering
- Failing to acknowledge the wide distribution of nociceptors, and mechanisms and thresholds for activation being of sufficient intensity to threaten tissue integrity
- Omission of key domains asked for, particularly spinal cord transmission

5. Describe how the kidney handles and regulates potassium throughout the nephron.



48% of candidates achieved a pass in question 5.

Learning Outcome(s)

- | | |
|------------|---|
| BT_PO 1.72 | Describe the function, distribution and physiological importance of ... potassium ... |
| BT_PO 1.73 | Describe the mechanisms involved in the maintenance of fluid and electrolyte balance |

Answer Domains

- Filtration
- Reabsorption
- Secretion
- Regulatory mechanisms

Additional credit and features of better answers

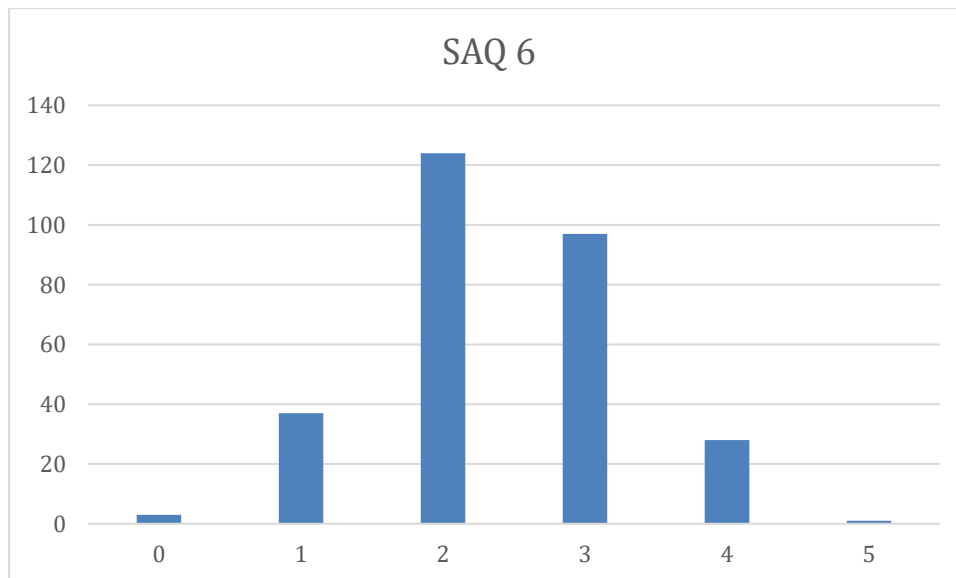
- Details on the sites and percentages of potassium reabsorption
- Demonstrated understanding that potassium secretion is the main way by which the body maintains potassium homeostasis
- Outlining the role of aldosterone as it relates to potassium independently of the renin-angiotensin-aldosterone-system

Problems and recurring errors

- Diagrammatic representation of the nephron, without relating this to potassium handling
- Omission of information on potassium secretion and factors controlling this
- Imprecise, inaccurate or confused use of terms (secretion/excretion/absorption)
- Inaccurate information regarding different ion channels, their location on the cell membrane and the direction of potassium movement
- Producing an answer on diuretics without linking this to potassium flux at all
- Limiting the discussion of aldosterone to RAAS activation, without consideration of hyperkalaemia

6. Outline the local, neural and hormonal mechanisms which influence the rate of gastric emptying.

(Do not include pharmacology)



43% of candidates achieved a pass in question 6.

Learning Outcome(s)

BT_PO 1.107 Outline the control of gastric motility and emptying ...

Answer Domains

Mechanisms influencing the rate of gastric emptying as given in the question

- Local
- Neural
- Hormonal mechanisms influencing the rate of gastric emptying

Additional credit and features of better answers

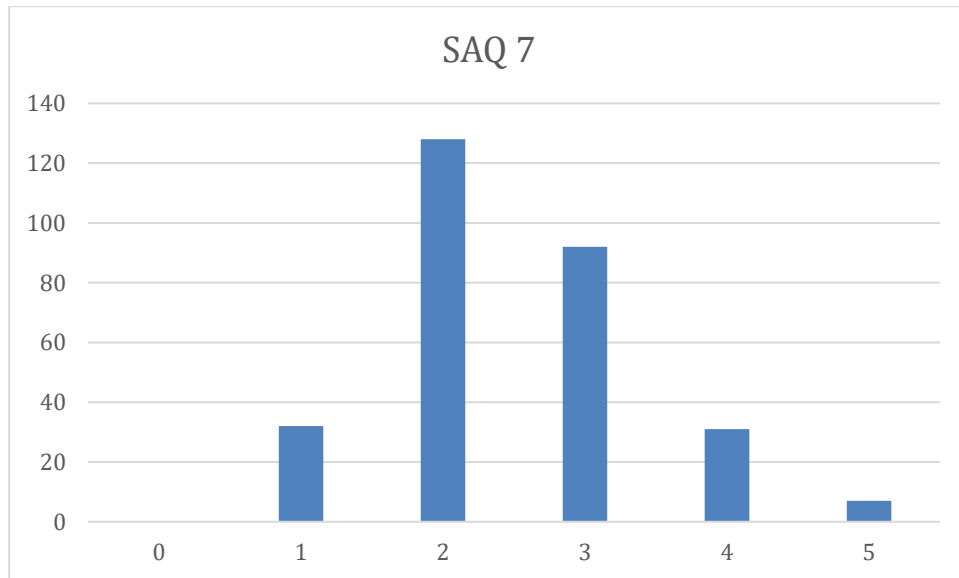
- Clearly understood the mechanisms promoting (mostly gastric) and impeding (mostly duodenal) gastric emptying and showed understanding of how this relates to the nature of the food
- Linked clinical significance (fasting guidelines) to this understanding.
- The differing effects of solids vs liquids, and types of macronutrients on rates of emptying
- Highlighting the significance of duodenal feedback

Problems and recurring errors

- Omission of material more so than factual error, as given domains were provided, this made it difficult to pass if a reasonable attempt wasn't made on each.
- Of three domains the hormones section was the poorest performing
- Discussion of lower oesophageal sphincter tone instead of gastric emptying
- Adding pharmacological factors
- Long introduction without answering the question

7. a) Define the terms tolerance, dependence and addiction (30%)

b) Outline the mechanisms and clinical significance of opioid tolerance (70%)



45% of candidates achieved a pass in question 7.

Learning Outcome(s)

BT_GS 1.15 Define tachyphylaxis, tolerance, addiction, dependence and idiosyncrasy.
Describe mechanisms of tolerance

Answer Domains

- Definitions of tolerance, dependence and addiction
- Mechanisms and clinical significance of opioid tolerance

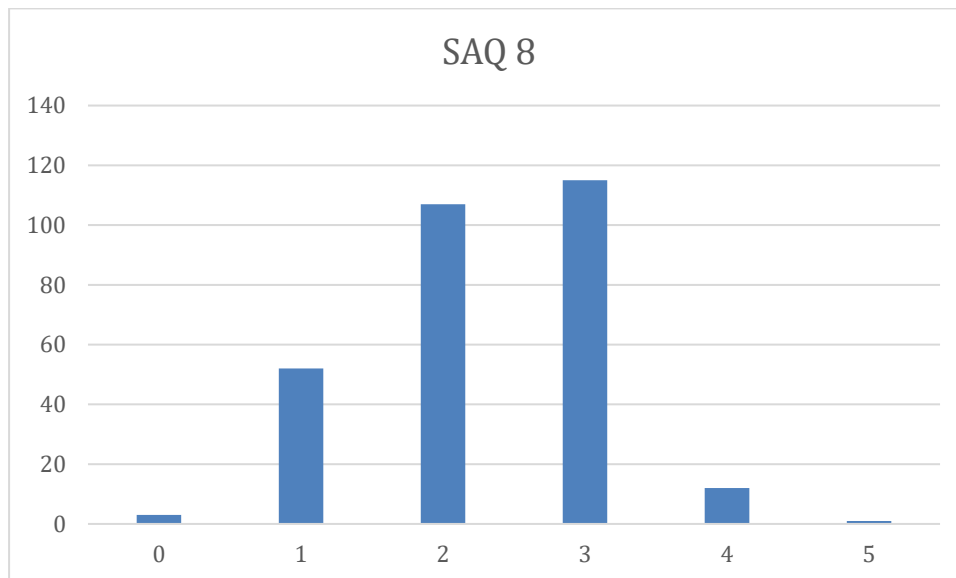
Additional credit and features of better answers

- Time course of the development and loss of tolerance
- Factors which increase the development of tolerance (potency, dosing interval)
- Differential tolerance to constipation and miosis
- Incomplete cross-tolerance between opioids and explanation of opioid rotation

Problems and recurring errors

- Confusing the definitions of dependence and addiction
- Listing rather than outlining mechanisms
- Errors in pharmacokinetic mechanisms of opioid tolerance
- Confusing the terms potency and efficacy
- Confusion about which side effects of opioids are subject to tolerance

8. Explain the concentration effect and second gas effect in relation to the wash-in of sevoflurane with nitrous oxide.



44% of candidates achieved a pass in question 8.

Learning Outcome(s)

- | | |
|------------|--|
| BT_GS 1.24 | Describe the uptake, distribution and elimination of inhalational anaesthetic agents |
| BT_GS 1.27 | Describe the pharmacology of nitrous oxide |
| BT_GS 1.28 | Describe the comparative pharmacology of – nitrous oxide, sevoflurane, desflurane |

Answer domains

- Concentration effect with regard to sevoflurane use with nitrous oxide
- Second gas effect with regard to sevoflurane use with nitrous oxide

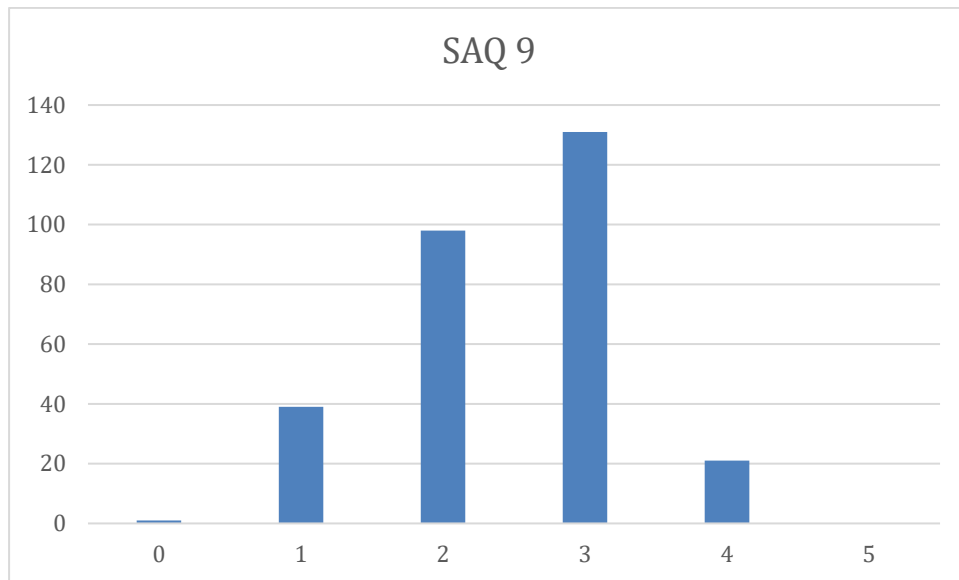
Additional credit and features of better answers

- Comparison of speed with use as a single agent
- Consideration of wash-in – a definition or graph, and factors affecting it with regard to inhalational agents

Problems and recurring errors

- Incorrect and imprecise description of the processes
- Misunderstanding how physicochemical properties relate to clinical characteristics of sevoflurane and nitrous oxide
- Failure to address the wash-in part of the question (ie during the initial phase of inhalational anaesthesia)

- 9. Discuss the use of ketamine for intravenous induction in hypovolaemic shock using pharmacokinetic and pharmacodynamic principles. Include and justify appropriate doses in your answer.**



52% of candidates achieved a pass in question 9.

Learning Outcome(s)

BT_PM 1.27 Describe the pharmacology of ketamine

Answer domains

- Applied physiology for bolus kinetic data for ketamine – reduced central volume, increased proportional cerebral blood flow and impact on drug distribution
- Applied physiology for pharmacodynamic data for ketamine – indirect and direct CVS effects of ketamine, and context of depleted endogenous catecholamine stores; and increased CNS sensitivity in shock
- Dosing – euvoalaemic and comparative shock dosing with justification

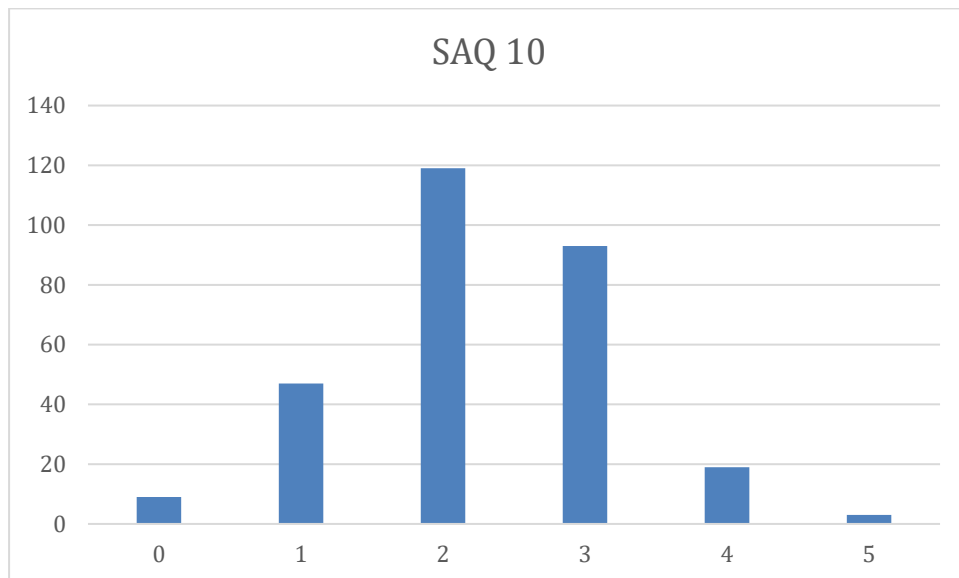
Additional credit and features of better answers

- Explaining the balance between indirect and direct pharmacodynamic CVS effects
- Mitigation strategies for likely sequelae of ketamine bolus dosing

Problems and recurring errors

- Failure to adequately or accurately address the dosing and justification part of the question
- Safety breaches with regards to dosing used
- Including details of ketamine pharmacology irrelevant to the question asked (eg NMDA receptor interaction), or without relating it to shock (eg standard drug data with no application to shock)
- Incorrect information regarding the pharmacodynamics of ketamine

10. Discuss the factors affecting duration of action of a local anaesthetic block to a major peripheral nerve.



40% of candidates achieved a pass in question 10.

Learning Outcome(s)

BT_RA 1.3 Discuss the pharmacology of local anaesthetic agents

Answer Domains

- Drug factors
- Patient/nerve factors

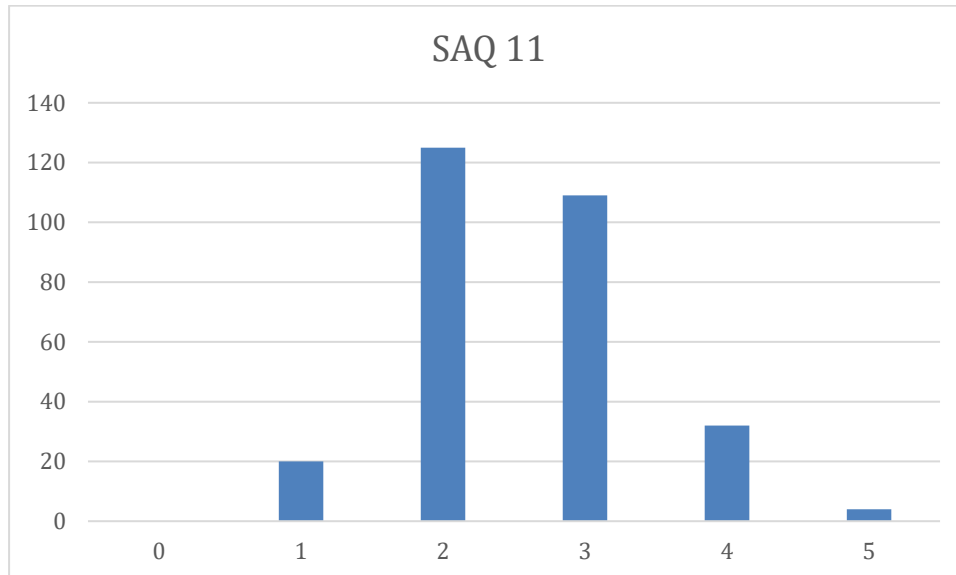
Additional credit and features of better answers

- Detailed descriptions of why each of the listed factors affected duration of action
- Considering special populations such as neonates, pregnancy, and the elderly, and how duration of action is affected in these populations

Problems and recurring errors

- Lengthy definitions of local anaesthetics and their mechanism of action which did not attract any marks
- Discussing factors that increased speed of onset, but have no effect on duration of action
- Missing one of the domains entirely in the answer (usually patient factors)

11. Compare and contrast the pharmacology of atracurium and rocuronium.



50% of candidates achieved a pass in question 11.

Learning Outcome(s)

- | | |
|------------|--|
| BT_GS 1.36 | Describe the mechanism of action and pharmacokinetics of neuromuscular blocking agents |
| BT_GS 1.37 | Describe the pharmacological differences between neuromuscular blocking agents and the clinical importance of these differences. |

Answer Domains

- Pharmaceutics
- Pharmacokinetics, including mechanism of offset of each agent, and reversal options
- Pharmacodynamics, including dose, expected onset times and durations, mention of ED95.

Additional credit and features of better answers

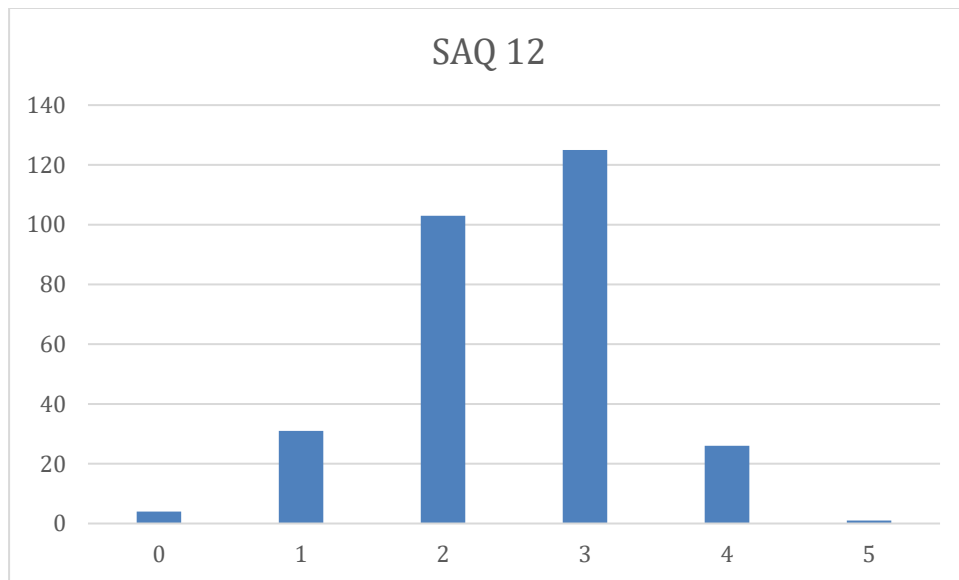
- More detailed discussion of clinical implications, and systemic effects
- Relationship between potency and onset
- Clinical implications that might determine the choice to use one agent over another

Problems and recurring errors

- Many candidates clearly had much greater knowledge about rocuronium than atracurium
- Listing pharmacological data without attempt to relate this to how we use these agents.
- Omitting key differences which determine choice between agents
- Discussion about cisatracurium rather than atracurium.
- Failure to contextualise histamine release seen with atracurium. Haemodynamic effects of atracurium were often overstated
- Factual errors regarding reversal options and anaphylaxis implications for each agent.

12. Outline the pharmacology of amiodarone using the following headings;

- a) mechanism of action**
- b) pharmacokinetics**
- c) adverse effects**
- d) adult cardiac arrest indications and dosage**



52% of candidates achieved a pass in question 12.

Learning Outcome(s)

BT_PO 1.57 Describe the pharmacology of amiodarone. Outline the pharmacology of other antiarrhythmic agents

Answer Domains

- As given in the question

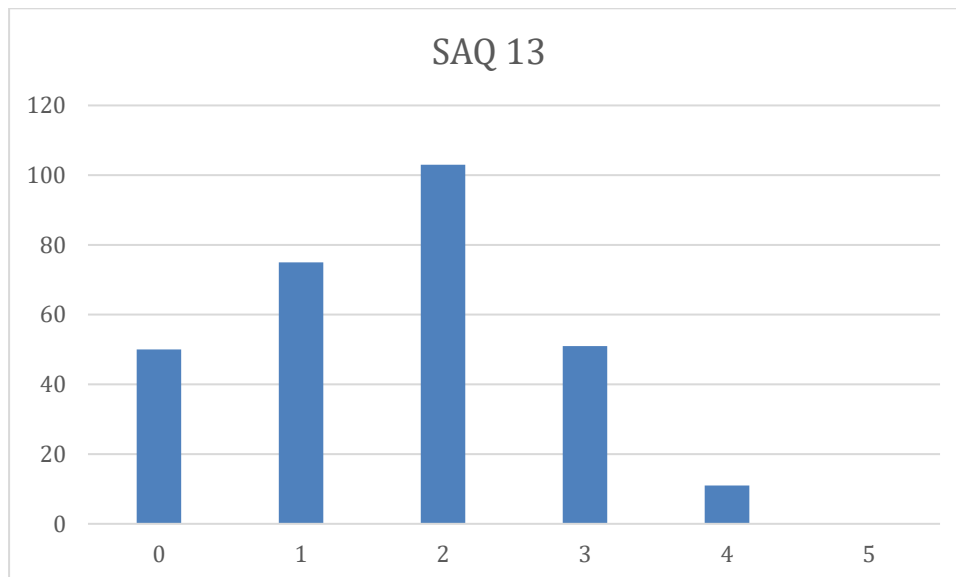
Additional credit and features of better answers

- Demonstrated extended knowledge such as relating ion channel effects of amiodarone to cardiac action potential, or why arrhythmias are suppressed
- Specific data on distribution, protein binding, volume of distribution and half-life presented in an organised fashion
- Structured section on adverse effects with inclusion of haemodynamic and respiratory effects

Problems and recurring errors

- Classification of amiodarone without elaborating on mechanism of action
- Irrelevant material about other anti-arrhythmics
- Lack of detail regarding pharmacokinetics
- Incorrect dosing or inappropriate indications in adult cardiac arrest

- 13. a) Draw and label a Mapelson F breathing circuit (Jackson-Rees modification of Ayre's T-piece)**
b) Explain how it works during spontaneous ventilation breathing 100% oxygen



21% of candidates achieved a pass in question 13.

Learning Outcome(s)

BT_SQ 1.13 Describe and classify breathing systems used in anaesthesia and resuscitation. Evaluate their clinical utility and hazards associated with their use

Answer Domains

- Labelled diagram
- Function during inspiration
- Function during expiration
- Other supporting facts

Additional credit and features of better answers

- In general answers which described function during inspiration, and separately considered function during expiration were better able to demonstrate an understanding of how the breathing system worked.

Problems and recurring errors

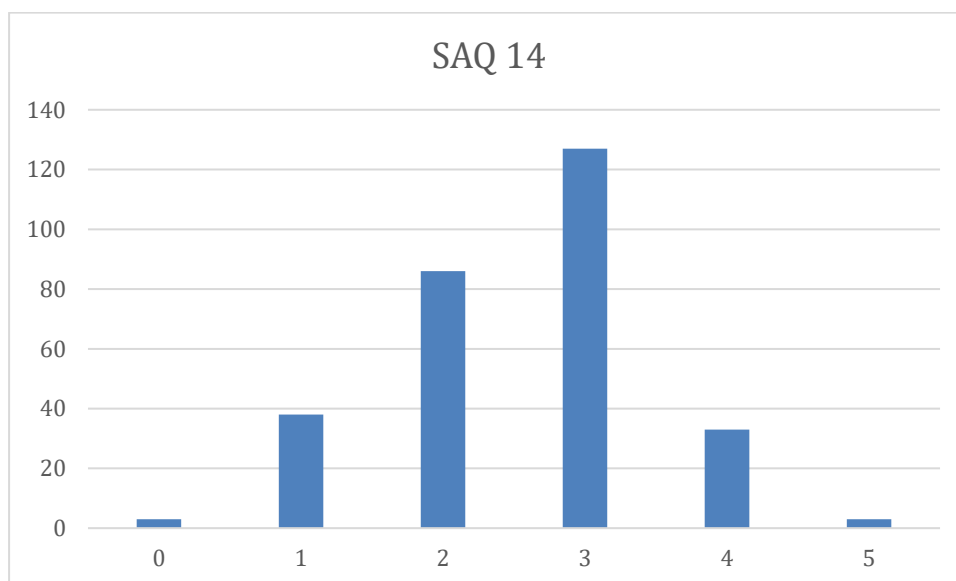
- Diagrams of other breathing systems
- Incorrectly drawn diagrams resulted in inaccurate explanation of function
- Providing information about the breathing system (eg advantages and disadvantages), without explaining how the system works

14. Outline the pharmacology of the following diabetic medications

- a) glipizide
- b) metformin
- c) empagliflozin

Using the following headings

- **mechanism of action**
- **side effects**
- **perioperative cessation and recommencement**



56% of candidates achieved a pass in question 14.

Learning Outcome(s)

BT_PO 1.90 Outline the pharmacology of: Insulin preparations oral hypoglycaemics

Answer Domains

- As given in the question

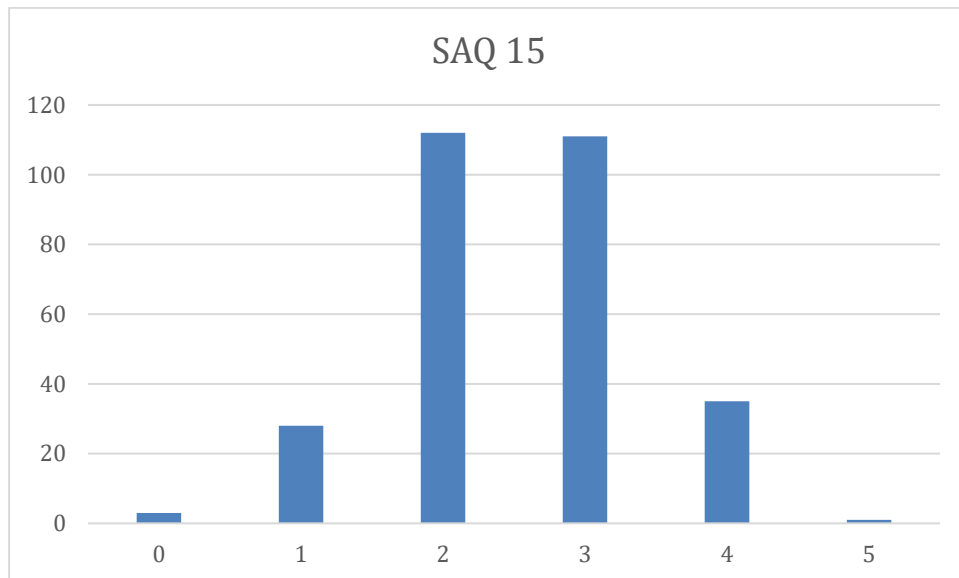
Additional credit and features of better answers

- Succinct outline of mechanism of action for each drug. Complicated cellular-level details were not required for a pass
- Outline of side effects with greatest emphasis on most clinically relevant perioperative complication for each drug

Problems and recurring errors

- Assigning mechanism of action from a different drug class
- Incorrect mechanism of action for metformin
- Omitting requested domains in the answer
- Use of generic statements (eg “follow local guideline”) without demonstrating any knowledge of the pharmacological basis of perioperative recommendations

**15. Explain the physical principles by which an ultrasound can generate a two dimensional image.
(Do not include Doppler)**



51% of candidates achieved a pass in question 15.

Learning Outcome(s)

BT_RA 1.8 Describe the principles of ultrasound imaging

Answer domains

- Ultrasound transducer properties – clinically used range of transducers
- Physics of ultrasound – generation and detection of ultrasound
- Interaction of ultrasound beam with tissues – waves travel through tissues, returning waves are used in image generation, time for and intensity of returning signal used in image generation

Additional credit and features of better answers

- Clinical application of transducer selection
- Clinical application of physics of ultrasound in image generation
- Clinical application of ultrasound wave-tissue interactions

Problems and recurring errors

- Failure to adequately address key domain of physics of ultrasound
- Imprecise and inadequate description of interaction of ultrasound beam with tissues

VIVAS

The viva examination comprises three 20 minute stations. Within each station there are 4 x 5 minute vivas. Each candidate receives 12 separate vivas, of equal mark value. The marks from the viva examination contribute 50% of the total final score.

The pass mark for each individual 5 minute viva is 3/5, marked using a holistic marking rubric. The allocation of marks is based largely on demonstrating knowledge and understanding across a number of domains. Standard setting for the vivas is achieved by varying the expectation for 3/5 for each viva to match a reasonable 'minimum level of competence', depending on the topic. Errors or contradictory statements do contribute to a reduction in marks. Progression through enough material within the viva is also taken into account, as well as the extent of any prompting.

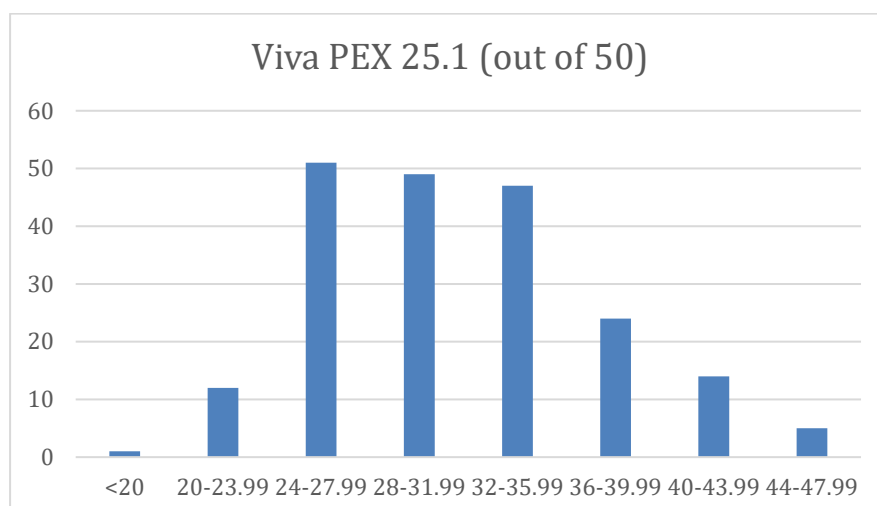
At each station there are 2 examiners. Each examiner independently submits marks for each viva. In the situation where one examiner awards 3/5, and the other awards 2/5 then a feedback form is completed and the viva is attributed a 'borderline' rating. This feedback sheet is separate to the actual mark collation sheet, and in the above situation the candidate will achieve a 5/10 for that viva.

The viva questions are all blueprinted to the curriculum. At each sitting of the exam, the vivas are selected to avoid overlap with material already examined in the SAQ paper. Therefore, the SAQ and viva components are planned together for each exam sitting. Within the 12 vivas, each candidate can expect to be examined on respiratory and cardiovascular physiology, core anaesthetic pharmacology, and a range of other topics.

Further information about the viva exam is available on Learn@ANZCA in the **Planning for Success** online examination resource

The pass mark for the Viva is 50%, and the viva invitation mark is 40%. For the 25.2 exam the median scores for different groups is shown below;

Group	Median score
Invitees (213)	32/50
Overall successful (175)	33/50



90% of invited candidates achieved a pass in the viva examination.

VIVA TOPIC AREAS

In the 25.1 Examiners report some of the “Essential” areas of the curriculum were outlined including;

- Respiratory physiology
- Cardiovascular physiology
- Inhalational anaesthetic agents
- IV anaesthetic agents
- Monitoring and measurement principles, and equipment

Within these broad topics are sub-topics which examiners consider as distinct, and hence are blueprinted to appear in the SAQ and Viva exams of the same year without there being any overlap in examination coverage. These sub-topics are based on Appendix 2 of the curriculum and were listed in the 25.1 Examiners Report. In a similar way the sub-topics in three other core areas of the curriculum are listed below. The list below reflects how they appear in the viva and SAQ exam mapping.

LOCAL ANESTHETICS	LA - classification, physicochemical, structure
	LA - onset
	LA - duration
	LA - differential block
	LA - toxicity
NMBD	NMBD - classification, physicochemical, structure
	NMBD - Suxamethonium (PK, PD, side effects)
	NMBD - PK (comparative)
	NMBD - PD (comparative)
	NMBD - differential block, residual block, side effects
	NMBD - reversal agents
OPIOIDS	Opioids - classification, physicochemical, structure
	Opioids - MOA, receptors, dose equivalence
	Opioids – PK (comparative), bolus kinetics, infusion kinetics
	Opioids - PD (comparative)
	Opioids – adverse effects, tolerance, dependence, addiction

VIVA OPENING QUESTIONS – previously published

After consideration, the opening questions of the Vivas have been removed from this report. Instead the sub-topics provided above (and in the 25.1 Examiners Report) are given, as they form the basis of the SAQs and vivas which are prepared, templated and delivered.

NOTICES TO CANDIDATES AND SUPERVISORS

Appendix 2 – Study Guide for the Primary Exam

The learning outcomes (LOs) in Appendix 2 are grouped by body systems and topics, in a format which is intended to be a study guide for teaching and learning. All exam questions are blueprinted to the curriculum.

Library

a) Prescribed Reading List

<https://libguides.anzca.edu.au/primary>

The layout of the official reading list has been revised to improve clarity. Important texts are indicated by a “highly recommended” tag, and a direct link to Appendix 2 is also now available from this page. For some texts, only a few chapters are needed for the primary exam and these are specified.

Some topics are only superficially covered in the textbooks and candidates are advised to draw on many resources as they study, in order to adequately cover the LOs. All questions are blueprinted to the curriculum.

b) Other Resources

<https://libguides.anzca.edu.au/anaestrain/assess/primary-revision>

Useful revision aids can be accessed from this page (especially for MCQ and viva questions).

Links to a number of websites are also listed on this page. These are **not** endorsed by the primary examination sub-committee, and may contain factual errors. Non-peer-reviewed resources must be used with caution.

Access to SAQ Paper

Within the primary exam section of *Learn@ANZCA*, a new tab has been created for the SAQ paper. The SAQ paper is being uploaded soon after each sitting of the written exam, in order to assist consultants and tutors with planning viva practise.

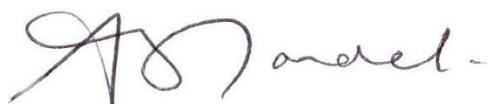
2025.2 COURT OF EXAMINERS

The primary exam committee would like to acknowledge the volunteer services of the examiners who marked a question and/or conducted vivas for this exam. We also acknowledge their employers and families who support them being away from their other commitments for the better part of a week during the viva exam.



Abhijeet Tandel (Chair)	Gosford Private Hospital	NSW
Celine Baber (Deputy Chair)	St John of God Hospital Subiaco	WA
Aaron Victor	Royal Darwin Hospital	NT
Adrian Lee	University Hospital Geelong	VIC
Alan Crowther	Waikato Hospital	NZ
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Alpha Tung	Auburn Hospital	NSW
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Ann-Marie Stevenson	Taranaki Hospital	NZ
Anthony Baird	Northeast Health Wangaratta	VIC
Anthony Hade	Royal Brisbane and Women's Hospital	QLD
Barbara Rodriguez	Monash Health	VIC
Benedict Waldron	The Alfred Hospital	VIC
Bryan Cook	Lismore Base Hospital	NSW
Catherine Goddard	Sir Charles Gairdner Hospital	WA
Chrissie Jenkins	Canberra Hospital and Wollongong Hospital	NSW
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Chrissie Moule	The Queen Elizabeth Hospital	SA
Courtney Roche	Townsville Hospital	QLD
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David Fahey	Royal North Shore Hospital	NSW
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Dean Dimovski	University Hospital Geelong	VIC
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Jakob Chakera	Joondalup Health Campus	WA
James Clark	Dunedin Hospital	NZ
Jeff Hoskins	Waikato Hospital	NZ
Jessie Ly	Westmead Hospital	NSW
Joyce Wong	The Tweed Valley Hospital	NSW
Julia Coldrey	Royal Adelaide Hospital	SA
Julie Chan	Western Health	VIC
Katelyn Priestler	Royal Adelaide Hospital	SA
Kim Heus	North Shore Hospital	NZ
Lisen Hockings	The Alfred Hospital	VIC
Mervyn Atkinson	Flinders Medical Centre	SA
Melinda Ford	Canberra Hospital	ACT
Michelle Harris	Royal Adelaide Hospital	SA
Nanki Singh	Pacific Anaesthesia	NSW
Natalie Marshall	Canberra Hospital	ACT
Nicholas Randall	Middlemore Hospital	NZ
Nicholas Harrington	The Queen Elizabeth Hospital	SA
Peter Schuller	Cairns Hospital	QLD

Quazi Siddiqui	Surgical Treatment and Rehab Service	QLD
Rebecca Branch	Middlemore Hospital	NZ
Rebecca Ruberry	Ipswich Hospital	QLD
Ritu Arora	Ipswich Hospital	
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Ruchi Lawrie	Westmead Hospital	NSW
Sam Lumb	Royal Adelaide Hospital	SA
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Simone Malan-Johnson	Caboolture Hospital	QLD
Stephen Williams	Westmead Hospital	NSW
Stuart McKown	Canberra Hospital	ACT
Tom Burrows	Waikato Hospital	NZ
Tyron Crofts	Frankston Hospital	VIC
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Dr Abhijeet Tandel
Chair, Primary Examination



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